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Correct Answer Show

## Practice Exam - 1

### Question 73 of 117



A 62 year-old man with ischemic cardiomyopathy and diabetes mellitus status post HeartMate 3 LVAS as bridge-to-transplant 1 year ago, listed status 4 for transplant as blood group O, presents with 2 weeks of malaise, fevers, and dull upper abdominal discomfort. He had a driveline exit site infection 2 months ago with methicillin-resistant *Staphylococcus aureus* (MRSA) that resolved with oral doxycycline and ciprofloxacin. Abdominal exam is notable for slight erythema and thin yellow drainage at the driveline exit site without induration. VAD hum is normal. Device interrogation reveals normal power and flow at 5000 rpm. Labs are notable for leukocyte count of 14, hemoglobin 11.5 mg/dl, platelets 256, creatinine 1.3 mg/dL. Blood cultures are pending. He is started on intravenous vancomycin and cefepime. CT scan shows a 3.3 x 2.1 x 6.5 cm fluid collection adjacent to the pump with soft-tissue stranding near the internal driveline.

What is the next best treatment?

- ☒ **A.** Surgical washout and debridement
- ☐ **B.** Targeted intravenous antibiotics for 6 weeks
- ☐ **C.** Switch to oral antibiotics if blood cultures are negative
- ☐ **D.** Status upgrade for urgent transplant

#### Commentary

#### References

**Testing Point:** The Board-certified AHFTC specialist should know how to treat pump pocket infection.

**Correct Answer:** A. Surgical washout and debridement.

**Rationale:** This patient has a pump pocket infection with abscess formation, as evidenced by fever, leukocytosis, persistent driveline drainage, and CT imaging demonstrating a fluid collection with surrounding soft-tissue stranding adjacent to the pump and internal driveline. Although he was previously treated for a superficial MRSA driveline infection, *Staphylococcus aureus*, particularly methicillin-resistant strains, can form biofilms on device surfaces, limiting antibiotic penetration and predisposing to persistent or recurrent infection. Therefore, adequate source control with surgical washout and debridement, with possible wound vacuum-assisted closure, is necessary.

Answer B: Prolonged intravenous antibiotic therapy will likely be required, but antimicrobial therapy alone is insufficient in the presence of a deep device-associated abscess without adequate source control.

Answer C: Even if blood cultures are negative, a pump pocket abscess remains a deep-seated device infection and is unlikely to respond to oral antibiotics alone.

Answer D: This patient is not an immediate candidate for transplantation while he has an active, untreated device infection and possible bacteremia (cultures pending). Although transplant with device explant may ultimately provide definitive treatment in select cases, infection control must first be achieved.

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